

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Podophyllotoxin and Imiquimod for the treatment of Genital Warts

Summary of NICE / NICE CKS Guidelines for Genital Warts

Overview

- **Genital warts (caused by HPV types 6 and 11):** The most common viral sexually transmitted infection (STI). Non-oncogenic HPV types. Warts can be treated but virus may persist; recurrence is common.
- **Natural history:** Lesions may regress spontaneously, persist indefinitely, or recur after treatment.

Assessment

- **Clinical diagnosis:** Visual examination of visible warts on anogenital area (external genitalia, perianal region). Appearance ranges from smooth to keratinized, single to multiple lesions.
- **Sexual history and STI screening:** Number of sexual partners, barrier contraception use, concurrent STIs, immunocompromised status.
- **Cervical screening status (women):** Ensure up to date with cervical cytology as appropriate for age and HPV history.
- **Wart characteristics:** Number, size, location (external vs internal), keratinization, and patient preference for treatment modality.

Management

- **Patient-applied treatments:** Imiquimod 5% cream (immune modifier) or podophyllotoxin 0.5% solution / 0.15% cream (cytotoxic agent) for external warts.
- **Treatment choice:** Based on wart location, number, keratinization, size, and patient preference. Podophyllotoxin for small external warts; imiquimod for larger or keratinized warts.
- **Clinic-applied treatments:** Cryotherapy or trichloroacetic acid (TCA) available for alternative approaches.

Red Flags / Specialist Referral

- **Pregnancy:** Avoid treatment; defer until after pregnancy. Specialist referral if internal warts or complications.
- **Immunocompromised patients:** Higher risk of extensive disease, poor response to treatment, and recurrence. Monitor closely; may require specialist input.
- **Suspicious lesions:** Atypical appearance, bleeding, ulceration, or concern for malignancy. Biopsy to exclude squamous cell carcinoma or other pathology.
- **Internal warts:** Urethral, vaginal, cervical, or rectal warts require specialist assessment and treatment.

Patient Group Direction

for the supply/administration of

Podophyllotoxin 0.5% solution / 0.15% cream (Warticon/Condyline)

for the treatment of

Genital Warts

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of small external genital warts. Patient-applied cytotoxic agent suitable for visible, non-keratinized external warts.
Inclusion criteria	Adults aged 18 years and over Visible external genital or perianal warts (not internal: urethral, vaginal, cervical, rectal) Small warts suitable for patient application (typically <50 warts or treatment area <10cm²) Able to identify warts and apply treatment to warts only (not healthy skin) Able to give informed consent
Exclusion criteria	Hypersensitivity to podophyllotoxin Pregnancy Breastfeeding

	Internal warts (urethral, vaginal, cervical, rectal) Open wounds or broken skin at application site Large treatment area (>10cm²) or >50 warts
Cautions	Teratogenic: strict contraception required if any risk of pregnancy Avoid contact with healthy skin: apply petroleum jelly to surrounding area as protective barrier before treatment Do not exceed maximum: 50 warts per session or treatment area 10cm² Solution: flammable; keep away from heat and ignition sources Wash hands immediately after application Avoid sexual contact while treatment applied (at least until washed off)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Podophyllotoxin 0.5% solution OR Podophyllotoxin 0.15% cream
Legal category	POM
Storage	Below 25°C. Keep away from heat and light sources. Keep in original container.
Route/method of administration	Topical application directly onto warts (solution or cream)
Dose and frequency	Apply thin layer twice daily for 3 consecutive days (e.g., Monday-Wednesday morning and evening) Follow by 4 days rest (no treatment Thursday-Sunday) Repeat cycle: continue for up to 4-5 cycles (4-5 weeks total treatment) Review progress after 2 cycles; if warts persist, repeat for additional 2 cycles
Quantity to be administered and/or supplied	1 bottle of solution (15mL) OR 1 tube of cream (5g) per treatment cycle
Maximum or minimum treatment period	4-5 treatment cycles (each 3 days on, 4 days off); typically 4-5 weeks total.
Adverse effects	Very common: local irritation, erythema, erosion, scabbing, pain at application site Common: burning, bleeding, edema, pruritus at wart site

	Rare (if significant absorption): systemic toxicity including myalgia, arrhythmias, neuropathy Yellow card report suspected adverse reactions via https://yellowcard.mhra.gov.uk
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with podophyllotoxin or imiquimod cream.
Follow-up advice to be given to patient or carer	Wash your hands after application of cream or solution (unless treating your hands). Apply treatment only to warts, not to surrounding healthy skin. Use Vaseline or petroleum jelly around warts to protect healthy skin. For podophyllotoxin: apply twice daily for 3 consecutive days (e.g., Mon-Wed morning and evening), then rest for 4 days (Thu-Sun). Repeat for up to 4-5 cycles. For imiquimod: apply 3 times per week at bedtime (e.g., Mon/Wed/Fri), and wash off the next morning after 6-10 hours. Continue for up to 16 weeks. Avoid all sexual contact while cream or solution is on your skin. Use condoms consistently with partners, even if warts are treated, to reduce transmission risk.

	Discuss diagnosis and transmission risk with sexual partner(s); they may need screening and/or treatment. Complete the full treatment course even if warts shrink or appear to clear. Seek medical advice if warts worsen, spread significantly, or do not improve after completing treatment cycles. Report any severe local reaction, excessive pain, bleeding, signs of infection, or systemic symptoms (fever, severe headache) to your healthcare provider. Attend follow-up appointment to assess treatment response and plan any further management. HPV vaccination should be discussed, as it can help prevent other HPV types and recurrence. Pregnant women should inform their healthcare provider; treatment will be deferred until after pregnancy. Regular sexual health screening is recommended.
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Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Genital warts
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
1/11/25	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Imiquimod 5% cream (Aldara)

for the treatment of

Genital Warts

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of external genital and perianal warts, particularly larger or keratinized lesions. Immune response modifier activating local and systemic immunity.
Inclusion criteria	Adults aged 18 years and over External genital or perianal warts (not internal: urethral, vaginal, cervical, rectal) Able to identify warts and apply treatment correctly Able to give informed consent
Exclusion criteria	Hypersensitivity to imiquimod or any component of cream Internal warts (urethral, vaginal, cervical, rectal)

	Open wounds or broken skin at application site Pregnancy Breastfeeding
Cautions	Uncircumcised males: risk of phimosis (foreskin swelling). Advise careful application; if phimosis develops, cease treatment and refer to GP. Autoimmune conditions: may exacerbate. Monitor closely. Severe inflammatory reaction: temporarily cease treatment, consider rest period, or dose reduction Avoid application to large skin areas or non-wart skin: use petroleum jelly as barrier on surrounding healthy skin Wash hands thoroughly after application (unless treating hands) Avoid sexual contact while cream on skin (cream can transfer to partner)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Imiquimod 5% cream in sachets
Legal category	POM
Storage	Below 25°C. Protect from light and moisture. Keep in original container.
Route/method of administration	Topical application to external warts
Dose and frequency	Apply thin layer 3 times per week at bedtime (e.g., Monday, Wednesday, Friday) Wash off after 6-10 hours (e.g., next morning with soap and water) Continue treatment for up to 16 weeks Review progress at 8 weeks; if complete clearance, stop treatment
Quantity to be administered and/or supplied	12 sachets per dispensing (sufficient for 4 weeks at 3x/week dosing)
Maximum or minimum treatment period	Up to 16 weeks (continuous if tolerating well); review at 8 weeks for response.
Adverse effects	Very common (>10%): local erythema, erosion, excoriation, oedema, scabbing at application site Common (1-10%): local pain, burning, induration, itching, ulceration;

	systemic: headache, flu-like symptoms, myalgia, fatigue Uncommon (0.1-1%): fever, elevated liver enzymes, lymphadenopathy Rare (<0.1%): systemic immune-mediated events, SLE-like syndrome Yellow card report suspected adverse reactions via https://yellowcard.mhra.gov.uk
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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Follow-up advice to be given to patient or carer	Wash your hands after application of cream or solution (unless treating your hands). Apply treatment only to warts, not to surrounding healthy skin. Use Vaseline or petroleum jelly around warts to protect healthy skin. For podophyllotoxin: apply twice daily for 3 consecutive days (e.g., Mon-Wed morning and evening), then rest for 4 days (Thu-Sun). Repeat for up to 4-5 cycles. For imiquimod: apply 3 times per week at bedtime (e.g., Mon/Wed/Fri), and wash off the next morning after 6-10 hours. Continue for up to 16 weeks.

	Avoid all sexual contact while cream or solution is on your skin. Use condoms consistently with partners, even if warts are treated, to reduce transmission risk. Discuss diagnosis and transmission risk with sexual partner(s); they may need screening and/or treatment. Complete the full treatment course even if warts shrink or appear to clear. Seek medical advice if warts worsen, spread significantly, or do not improve after completing treatment cycles. Report any severe local reaction, excessive pain, bleeding, signs of infection, or systemic symptoms (fever, severe headache) to your healthcare provider. Attend follow-up appointment to assess treatment response and plan any further management. HPV vaccination should be discussed, as it can help prevent other HPV types and recurrence. Pregnant women should inform their healthcare provider; treatment will be deferred until after pregnancy. Regular sexual health screening is recommended.
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